

Anticholinergic Toxicity Secondary to Overuse of Topricin Cream, a Homeopathic Medication

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Case presentation

A 40-year-old man presented with family to the emergency department complaining of headache and blurry vision of one day's duration. His wife also noticed a change in mentation for two hours prior to presentation when he repeatedly asked the same questions and did not recognize family members. His past medical history included hypertension and dyslipidemia. His daily medications were pravastatin, metoprolol, enalapril, and non-steroidal anti-inflammatories for musculoskeletal pains. Social history and family history were non-contributory. Two days prior to presentation, he had developed severe pain in his left foot which he treated with over-the-counter Topricin (topical pain relief) cream. He applied the cream liberally without respecting the recommended dosage. The exact quantity and frequency were unknown.

On physical exam, his vital signs were significant for sinus tachycardia at 135 beats per minute but otherwise was normal without fever. He was lethargic, yet arousable; his orientation could not be assessed. His pupils were small and no gross focal motor deficits could be elicited. A stroke code was initially called in the emergency room due to his altered mental status, and a subsequent computed tomography scan was negative for any intracranial pathology. Thrombolytics were not given. Laboratory studies showed hypercholesterolemia with a low-density lipoprotein of 114 mg/dL and a creatine kinase (CK) initially of 98 IU/L, which subsequently rose up to 1,837 IU/L on repeat lab testing. A troponin level and creatine kinase muscle-brain (CK-MB) were within normal limits. Urine drug screen was negative. Blood urea nitrogen and creatinine were 29 mg/dL and 1.59 mg/dL, respectively.

In the emergency department, the patient was disoriented and incontinent to urine. A Foley was placed and drained 950 mL of clear urine in one hour, with his incontinence likely representing overflow. Lorazepam was administered for acute delirium. A toxicology consult was obtained. At that point, the possibility of anticholinergic toxicity secondary to the overuse of the Topricin cream was suspected, given the favorable complex of presenting signs and symptoms. A review of the ingredients revealed belladonna. Physostigmine was then recommended as a diagnostic tool. However, due to the recent use of a benzodiazepine and the lack of imminent need, a decision was taken to withhold. The patient was admitted for observation and intravenous (IV) hydration. On the second day, his mental status had returned to baseline, his kidney

functions improved, and his CK level approached normal limits. He was discharged home after extensive education regarding the use of over-the-counter and homeopathic remedies.